

Public Health Emergency Operations Center "COUSP DRC"

MVE-17 Incident Management System

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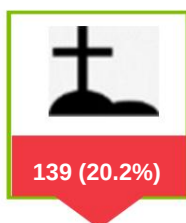
Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°28/MVB_11/06/2026

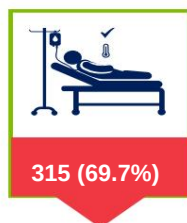
Affected provinces (3)	ITURI, NORTH KIVU & SOUTH KIVU
Affected Health Zones (29)	- Ituri (19/36): Aru, Aungba, Bambu, Bunia, Damas, Gety, Kilo, Komanda, Lita, Logo, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rimba, Rwampara, Tchomia, Kambala. - North Kivu (9/34): Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha, Masereka and Vuhovi. - South Kivu (1/34): Miti-Murhesa
Reporting date	June 11, 2026
Publication date	June 12, 2026



Total confirmed cases
for the 3 provinces



Cumulative deaths
among confirmed cases
and case fatality rate



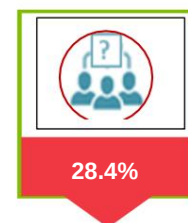
Patients in
isolation -
hospitalization



Cumulative number
of recoveries



Suspected cases of
the day



Contact tracing rates
for the 3 provinces

*Data is being harmonized to include confirmed cases still in the community as well as cases of escape

In the overall count: the number of cases in North Kivu has been revised downwards, from 44 to 40.

* *Suspicious deaths occurring in the community are subject to investigation

further investigation with a view to possible reclassification (non-cases, probable cases).

1. HIGHLIGHTS

- **Seventeen (17)** new confirmed cases, including 5 deaths, were reported as of June 11, 2026, in the health zones of **Bunia (6), Rwampara (5), Mongbwalu (4), and Nizi (2)** in Ituri Province. • Supervisory visits were conducted at Ebola Treatment Centers (ETCs) in the health zones of Butembo and Katwa by the National Emergency Operations Center (COUSP) and the National Integrated Management Team (IM) for the response to the 17th Ebola outbreak.
- Arrival of the Director General of the INSP and the multidisciplinary team (COUSP-WHO) in the Butembo area for strengthen the response actions in the said ZS and its surroundings.
- Receipt of a cardiac monitor and pool oximeter at the Rwampara CTE.

2. EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

Ituri province, located in the northeast of the Democratic Republic of Congo, shares a long and porous border with Uganda and South Sudan. For over two decades, it has faced a chronic humanitarian crisis linked to armed conflict and recurring population displacements. Its population is estimated at over 8 million, including more than one million internally displaced persons. The Mongbwalu health zone, considered the starting point of the epidemic, is located in the Djugu territory, 70 km from Bunia. This area is characterized by the presence of armed groups, frequent population movements towards Uganda, and numerous artisanal mining sites attracting migrant workers. In September 2018, Ituri province had already been affected by the 10th Ebola virus disease (EVD) outbreak, which was then raging in North Kivu. The current epidemic, caused by the Bundibugyo ebolavirus, was officially declared on May 15, 2026, by the Minister of Public Health. The three affected provinces have a total estimated population of nearly 15 million, with massive internal displacement and cross-border flows to neighboring countries.

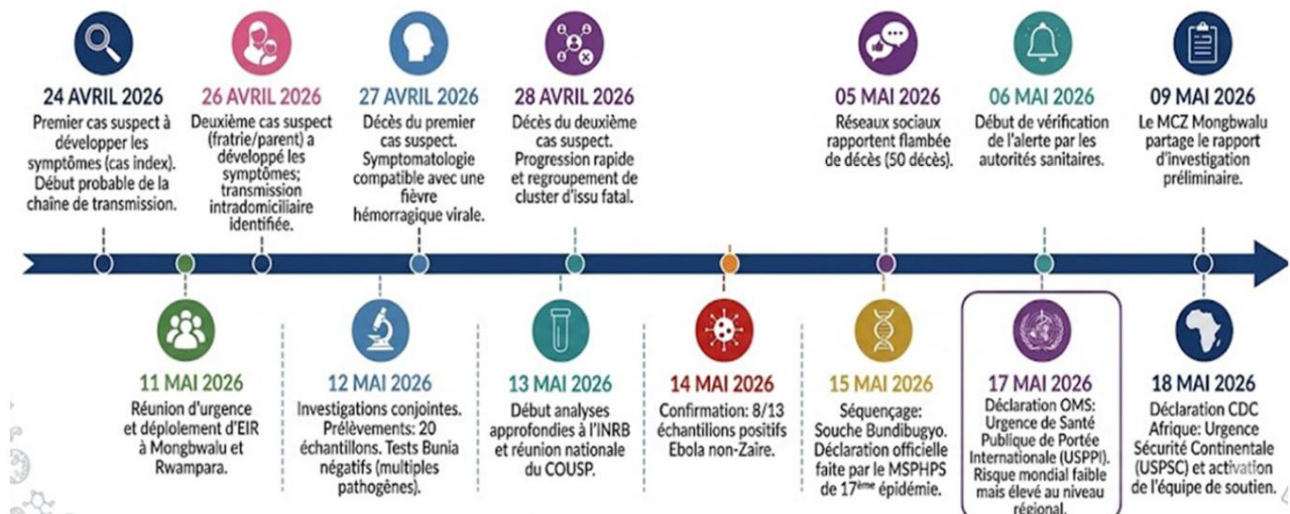


Figure 1. Chronology of events of the Ebola-Bundibugyo virus disease epidemic.

3. DETAILED EPIDEMIOLOGICAL ANALYSIS

3.1 Spatial distribution and active hotspots as of June 11, 2026

Province	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones	New confirmed cases (24h)
Ituri	646	114	17.6%	19 out of 36 (52.8%)	17
North Kivu 1	40	24	60.0%	9 out of 34 (26.5%)	0
South Kivu	3	1	33.3%	1 out of 34 (2.9%)	0
Total	689	139	20.2%	29 out of 104 (27.9%)	17

- No new health zones were affected by the end of the day on June 11, 2026. Thus, the number of affected health zones remains at 29/104 (section 3.1). Ituri province remains the epicenter with 93.8% of confirmed cases reported in the three affected provinces. North Kivu province remains the most affected in terms of case fatality rate (60.0%), while Ituri province shows a low case fatality rate (17.6%). South Kivu province has remained silent since May 26, 2026, the date of its last case.

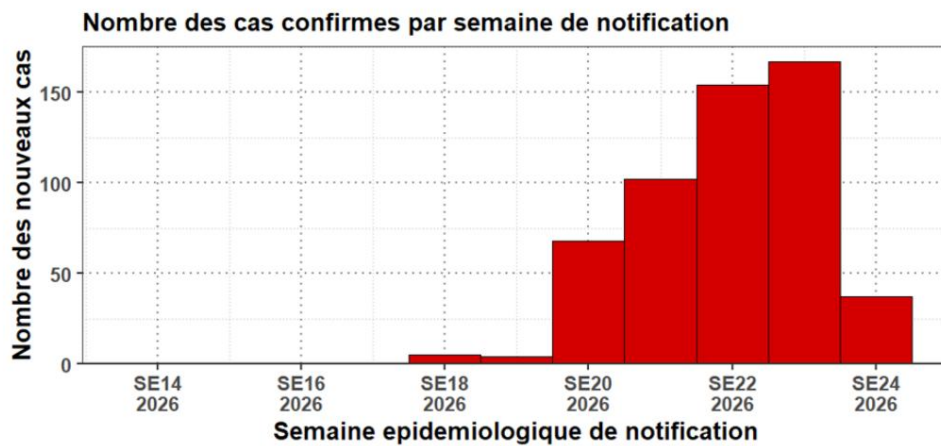
Confirmation has been notified.

3.2 Distribution by health zone (Cumulative as of 11/06/2026)

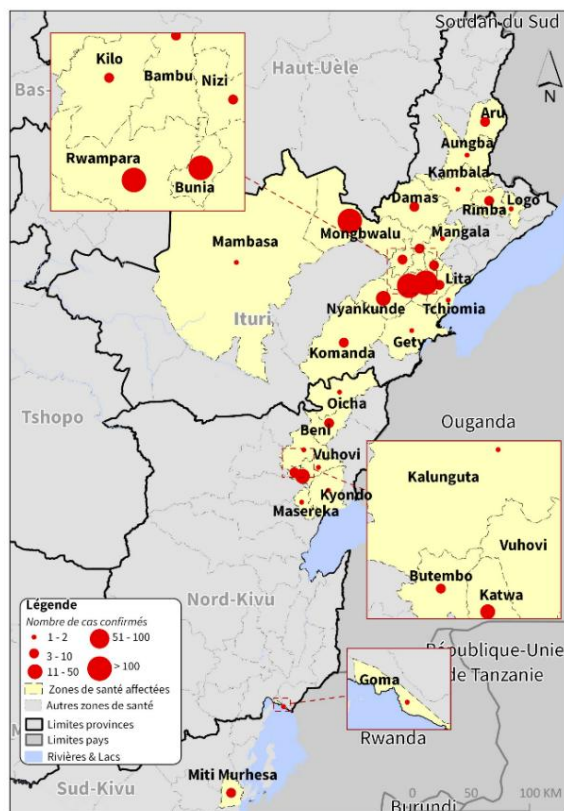
- Ituri Province (646 cases):** Bunia (191), Rwampara (142), Mongbwalu (136), Nyankunde (33), Nizi (9), Bambu (7), Lita (6), Kilo (4), Rimba (3), Aru (3), Damas (3), Komanda (3), Aungba (2), Mambasa (2), Mangala (2), Logo (2), Tchomia (2), Gety (1) and Kambala (1), (Ninety-four (94) additional cases are being ventilated.
- North Kivu Province (40 cases):** Katwa (16), Beni (9), Butembo (7), Oicha (2), Kyondo (2), Kalunguta (1), Goma (1), Masereka (1) and Vuhovi (1).
- South Kivu Province (3 cases):** Miti-Murhesa (3).

1 The data from North Kivu have been harmonized with the national SGI.

3.3 Time analysis: The number of confirmed cases is increasing week by week, indicating continued transmission of the disease within the community. A rapid geographical expansion of the epidemic is a real possibility if public health measures are not implemented quickly.



3.4 Epidemic mapping



Figures 2 & 3. Spatial distribution of confirmed Ebola cases by affected health zone and in the three affected provinces as of June 11, 2026

Table 1. Distribution of confirmed cases and deaths of Bundibugyo Ebola virus disease by health zone and province, Democratic Republic of Congo, as of June 11, 2026.

Province / Health Zone	Cumulative confirmed cases (n)	Cumulative nonconfirmed deaths (n)	Case fatality rate (%)
ITURI			
Bunia	191	17	8.9%
Rwampara	142	26	18.3%
Mongbwalu	136	52	38.2%
Nyankunde	33		3.0%
Bamboo	7	1	28.6%
Aru	3		33.3%
Kilo	4		25.0%
Nizi	9		11.1%
Mangala	2	2	0.0%
Damascus	3	1	0.0%
Aungba	2	1 1 0 0 1	50.0%
Gety	1	0	0.0%
Komanda	3	0	0.0%
Lita	6	0	0.0%
Logo	2	0	0.0%
Mambasa	2	1	50.0%
Rimba	3	0	0.0%
Tchomia	2	0	0.0%
Kambala			100.0%
Other ZS (undisclosed data)	1 94	1 10	10.6%
Ituri subtotal	646	114	17.6%
NORTH KIVU			
Katwa	16	9	56.3%
Blessed	9	5	55.6%
Butembo	7	5	71.4%
Oicha	2	2	100.0%
Kalunguta			100.0%
Kyondo	1		50.0%
Goma		1	0.0%
Masereka		1	0.0%
Vuhovi			100.0%
North Kivu subtotal	2 1 1 1 40	0 0 1 24	60.0%
SOUTH KIVU			
Miti-Murhesa	3	1	33.3%
South Kivu subtotal	3	1	33.3%
TOTAL	689	139	20.2%

4. RESPONSE ACTIONS BY PILLAR

4.1 Coordination

National / Ituri:

- Mission to assess coordination in Beni and Butembo as part of the response to the 17th epidemic.
- Since Wednesday, a preparation briefing on Ebola virus disease (EVD) has been underway for all staff at the Cinquantenaire Hospital in Kinshasa (site for the management of EVD cases in Kinshasa).
- Holding a working session (SGI-PTF-National Level Expert Coordination) in the Rwampara Health Zone to strengthen epidemiological surveillance aspects, particularly contact listing and follow-up.
- Supervisory visit by COUSP and the national IM in the CTEs (ZS of Butembo and Katwa).
- Working exchange between the Director General of INSP and the provincial team of the Butembo branch on strengthening response actions.
- Closure of training for national and provincial SGI members.

4.2 Epidemiological surveillance

Table 2. Management of epidemiological alerts (24h) as of June 11, 2026

Indicator		Ituri	North Kivu	South Kivu	Total
Alerts raised		143	266	10	419
Alerts investigated		101	266	10	377
Investigation rate		70.6%	100.0%	100.0%	90.0%
Alerts confirmed	Alive	33	61	10	104
	Deceased	12	52	0	64
Total suspected cases for today		45	113	10	168

A total of 419 **alerts** were received across the three affected provinces, of which **377 (90.0%)** were investigated. During the day of June 11, 2026, **168 suspected cases (64 deaths)** were recorded in the three affected provinces.

Table 3. Contact tracing of confirmed cases as of June 11, 2026

Province	Contacts under follow-up	Contacts viewed	Follow-up rate (%)
Ituri	(n) 4,782	(n) 830	17.4% ²
North Kivu	1037	700	67.5%
South Kivu	224	185	82.6%
Total	6043	1,715	28.4%³

- Continued validation activities for identified probable cases.
- Continued capacity building for health center teams and RECO of the health areas visited on the Ebola surveillance and community-based surveillance.

² Data on contact tracing from June 11, 2026 for the health zones of Rwampara and Bunia have not been made available even though they contain 55.6% of the contacts of the entire province.

³ This low overall rate is the result of unreported data in the Ituri province.

- 234 new contacts were listed in the health zones of Lita (115), Nizi (101), Bambu (14) and Kambala (4).

Update on checkpoint and point of entry (PoC/PoE) activities

Table 4. Monitoring of indicators at Points of Entry and Points of Control (PoE/PoC) in Ituri province, as of June 11, 2026

Indicators	Results
Proportion of activated PoCs	43.6% (17/39)
Number of past travelers % of	34,118
screened travelers	97.2%
% of travelers who washed their hands % of	98.1%
travelers who were aware	92.8%
Number of alerts notified	3
Percentage of alerts investigated or bodies swabbed within 24 hours	0
Number of validated alerts	3
% of results available in 24 hours	0%
Number of alerts transferred to the CT/CTE or for EDS	0

4.3 Laboratory

- **Ituri:** 35 new samples collected and analyzed (positivity 48.6%; n=17)
- **North Kivu: 172 samples pending in North Kivu (delay > 5 days).**

4.4 Infection Prevention and Control (IPC)

- **Ituri:** Start of training of 36 TS and 36 teachers on IPC/WASH in the Komanda Health Zone; continuation of training (Day 2) in IPC and EDS (Nyankunde Health Zone); briefing of 10 healthcare providers from 4 ESS on handwashing, bio-cleaning, waste management, wearing and removing PPE (Bambu Health Zone); briefing of 56 members (community leaders, group chiefs and IT) on Ebola and prevention measures; decontamination of 2 ESS (Marie Claire Clinic: Babenda and Rwankole Hospital) and 2 households of confirmed cases (Bigo and Lembabo Quarters); briefing of 3 healthcare providers from Komanda Health Center on handwashing.
- **North Kivu:** Evaluation of 4 healthcare facilities (FEPSI Hospital: 61%, Shangilieni Hospital: 48%, Kyaghala Hospital: 68%, and La Charité Dispensary: 15%); 2/2 facilities decontaminated (Katwa General Hospital and FEPSI Hospital); 46/56 service providers briefed on infection prevention and control (IPC) topics across 14/17 facilities; risk assessment of exposure for 15/36 service providers in healthcare facilities in the Katwa (Kyaghala Hospital) and Butembo (FEPSI Hospital, Shangilieni Hospital, and La Vérité Hospital) health zones (among PPLs: 11 at high risk, 3 at intermediate risk, and 1 at low risk); 26 ESS supported in the ZS of Goma, Karisimbi and Nyiragongo with 29 service providers briefed on the different themes of the PCI.
- **South Kivu:** Decontamination of 12 households and 9 toilets around a suspected case in the Kabushwa health district; support for the Emergency Response Team (donning and doffing personal protective equipment) for 6 samples taken, including 2 in the Kahungu health district, 1 in the Lwiro health district, and 3 in the Kabushwa health district; theoretical and practical briefing of 6 hygiene workers from the Kahungu health center on the "3 buckets" decontamination technique; support for the 2 health centers

(Kahungu and Kalagane) where 12 service providers were briefed on standard IPC precautionary measures in a hospital setting.

4.5 Holistic care

- Receipt of a heart rate monitor and oximeter from pools at the Rwampara CTE.
- Monitoring of construction work on nurseries and CTEs (CME, ISTM Nyankunde, HGR Rwampara and Elykia).
- Nutritional monitoring of patients in all CTEs.

Table 5. Patient movement within healthcare facilities (CTE/CT/CI) as of June 11, 2026

Indicator of	Ituri	North Kivu	South Kivu	Together
Patients in Bed (Day -1)	247	13		267
Total admissions (24h)	47	18	7 10	75
Exits (24h)				
Deceased (Suspects/Confessions)	7	0	0	7
Non-cases /	7	0	5	12
Recovered / Escaped (Suspects/Conf.)	4	0	0	4
Transferred to the General	4	0	0	4
Referral Hospital	22	0	5	27
(HGR) Total discharges Patients in isolation (End of Day)	272	31	12	315
including confirmed (NC+AC)				138
and suspected cases		6	1	177
Overall occupancy rate	131,141,77.9 % (272/349)	25 43.7% (31/71)	11 37.5% (12/32)	69.7% (315/452)

Mental health activities and psychosocial support

Table 6. Key indicators of mental health and psychosocial support activities as of June 11, 2026

KEY INDICATORS	ITURI	NORTH KIVU	SOUTH KIVU
New confirmed cases admitted to the CTE who benefited from SMSPS interventions	2 (25%)	0	0
Confirmed cases currently being monitored that have benefited from SMSPS interventions	47 (38.2%)	4 (60%)	1 (100%)
New suspected cases that benefited from SMSPS interventions	7 (14.9%)	2 (100%)	2 (100%)
Suspected cases under follow-up that have benefited from SMSPS interventions	12 (12.8%)	20 (90.9%)	9 (100%)
Laboratory results have been announced	12	0	0
Including positive ones	5	0	0
Number of children separated in daycare who benefited from SMSPS interventions	1	4 (100%)	0
Number of children separated in the community who benefited from SMSPS interventions	16	3	0
Number of accompanying persons at the CTE who benefited from SMSPS interventions	23	12	0
Number of PPLs who benefited from SMSPS interventions	9	17	

Number of household and community members who benefited from SMSPS interventions	45	6	59
Number of EDS supported	2	7	0

4.6 Continuity of services.

- Evaluation of 5 ESS in the Nizi health zone.
- Implementation of recovery plans for social and solidarity economy enterprises that have already been assessed.

4.7 Risk communication and community engagement

North Kivu: 11,853 people sensitized in public spaces and strategic points, strengthening prevention and community confidence in support of Ebola response measures; 77 key community actors including 41 women and 36 men, including community leaders and influencers, committed to improving the dissemination of reliable messages and adherence to the response in their sermons; 18 radio stations produced 18 programs and 36 spots with massive dissemination of messages via these local radio stations on prevention, signs of Ebola and hand hygiene; 11 community alerts reported with the involvement of 368 community actors, thus strengthening surveillance and early warning in communities.

South Kivu: Advocacy with 45 leaders in the Katana health zone on their involvement in the fight against Ebola virus disease; holding of the CREC pillar meeting in the Mitimurhesa health zone on interventions to be carried out around Ebola cases; briefing of 10 CREC pillar members selected from different layers of the community in the Katana health zone.

Ituri: Evaluation of Ebola response activities in the Mongbwalu Health Zone under the chairmanship of the Djugu Territorial Administration; advocacy with the Mayor of the Shari Commune for his involvement in regulating prayers in the Balidja morgue enclosure not far from the CME/Nyankunde, Rwampara Health Zone.

Observation: Funeral prayers for the removal of lifeless bodies are organized within the grounds of this morgue and brass bands are used on this occasion.

Result: The Mayor promised to invite the owner of this morgue to question him in accordance with the instructions of the Governor of the province.

Support for the other pillars :

- **Surveillance** : investigation and listing of contact cases (Bunia Health Zone, BIGO Health Area);
- **IPC** : preparing families to participate in the EDS in the event of community deaths (Bunia Health Zone, Health Area Bankoko and Bigo).

Result: EDS completed.

4.8 Logistics and Security

- Receiving and delivering PCI supplies to the Rwampara Health Zone • Delivering CTE/CME-Bunia equipment.

4.9. Security

- The security situation in the city of Bunia (Bunia and Rwampara health zones) is relatively calm. However, the activity of armed groups in the territories of Djugu, Irumu, and Mambasa continues to limit humanitarian access in several affected or at-risk health zones.

4.10. PSEA (Prevention of Sexual Exploitation and Abuse)

- **In Ituri** : With the support of WFP's cooperating partners, 74 participants (31 women and 43 men) took part in activities organized in Bunia. In addition, 27 participants (10 women and 17 men),

including FHI360 staff, signed the code of conduct. Finally, a session with community leaders brought together 77 participants (30 women and 47 men).

- **North Kivu** : In Goma and Beni: organization of awareness sessions for the second wave of community relays and community leaders. 155 people took part in these activities, including 71 women, 53 participants (13 women and 40 men) of whom 21 signed the code (5F and 16M).
- **South Kivu** : PSEA briefing for actors involved in the response as well as healthcare providers from CS Kabushwa, a total of 42 people (13 women); PSEA briefing of 100 (31 women) teachers from ZS of Mitimurhesa (Lwiro) on EAS concepts, reporting mechanism followed by the signing of a code of good conduct.

5. MAIN CHALLENGES

Challenge	Impact	Action required
No. 1 Reluctance to perform post-mortem sampling (swabs)	Confirmation delay; underestimation of cases	Intensive community engagement; involvement of religious leaders
2 Insufficient capacity in standardized CTES despite the opening of Rwampara	Potential saturation of existing CTES	Acceleration of construction of CTE planned
4 Supply shortage Essential Generic Medicines (EGMs) in targeted General Respiratory Hospitals	Weakening of overall care	A plea for MEG funding
5 Insufficient PCI equipment (PPE, DLM, chlorine) in North Kivu	Nosocomial risk	Urgent supplies
6 Low increase in alerts in the 3 provinces	Delay in detection of case	Strengthening community surveillance
7 Insufficient financial resources for all pillars	Slowdown in operations	Resource mobilization (gap of USD 21.5 million)

6. PERSPECTIVES AND PRIORITY ACTIONS (24-72h)

Priority Action		Due date	
		Responsible pillar	
	Training of service providers on the use of the RADIONE PCR	Laboratory	Day 2
1 2	Briefing of service providers on Ebola virus disease (EVD) and tuberculosis control (TBC) surveillance in affected and high-risk areas	Monitoring/CREC	Day 3
3	Briefing of community health workers on Ebola virus disease surveillance and SBC (Ituri province)	Monitoring/CREC	Day 3
4	Supervised support from the Director General of INSP in the technical pillars	Coordination	Continue
5	Catch-up in contact tracing in Ituri remains weak.	Monitoring	Day 7
6	Strengthening infection prevention and control (IPC) measures and staff training in Nyankunde and other nosocomial areas	PCI	Day 2
7	Deployment of a standardized daily dashboard with KPIs per pillar	Coordination	Day 3
8	Plan to catch up on samples awaiting analysis in North Kivu	Laboratory	Day 2
9	North Kivu: discharge of the recovered case in Goma (Day 1), response to security incidents (ADF incursion), IPC training at Virunga General Referral Hospital, extension CREC for households		Day 2
10	South Kivu: allocation of 2 additional vehicles, installation of a kit Internet access in Miti-Murhesa, communication credits for IT/ECZS		Day 2



Evaluation mission of coordination in Beni and Butembo as part of the response to the 17th epidemic



Supervision of the construction work on the cabin PoC Dele in the Rwampara Health Zone



Start of the briefing for stakeholders at the 50th Anniversary University Hospital Center on Ebola Virus Disease as part of preparedness activities in Kinshasa

**FOR ALL
INFORMATION**

**ADDITIONAL,
PLEASE CONTACT**

For the National Institute of Public Health (INSP) of the DRC

The Director General of INSP

Dr. MWAMBA KAZADI Dieudonné

Email: diedonne.mwamba@insp.cd

Tel.: +243 816 040 145

The COU-SP Coordinator

Professor NGANDU Christian

Email: Christian.ngandu@insp.cd

Tel.: +243998091915

The Incident Manager

Professor Pierre AKILIMALI

Email: pierre.akilimali@insp.cd

Tel: +243815800288

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